PART 1: THE SELF-ADVOCACY "OPERATING MANUAL"

Concept: This is not a medical card. It is a "User Manual" for the student's brain. It bypasses emotional explanation and provides direct operational instructions to the receiver (Teacher, Manager, Peer). It should be credit-card sized or a saved image on a phone.

TEMPLATE: THE AuDHD INTERFACE CARD

[Front of Card]

MY NEURO-COGNITIVE INTERFACE (AuDHD)
I am not trying to be difficult. I am trying to function.

MY PROCESS	WHAT I NEED FROM YOU
I take instructions literally.	Please avoid metaphors or hints. Be explicit. Tell me clearly what "done" looks like.
My brain doesn't filter noise.	If I wear headphones, I am not tuning you out; I am tuning the background out so I can focus.
I process bottom-up (details first).	If I ask "Why?", I'm not challenging you. I need the logical framework before I can start the task.
My energy is all-or-nothing.	Please judge me by my final output, not by my process. My "lazy" looks different than my "recharging."

[Back of Card]

THE "RED ZONE" PROTOCOLS

If I go silent, shut down, or seem intensely irritable, I am in sensory/cognitive overload.

DO NOT:

- Ask me "What's wrong?" repeatedly.
- Demand eye contact.
- Touch me without permission.

PLEASE DO:

- Give me space and time (5-10 minutes).
- Lower your voice.
- Communicate via text/writing if urgent.

Thank you for helping me access my competence.

PART 2: RECOMMENDATIONS FOR DIAGNOSTIC PROFESSIONALS

AUDIENCE: Psychiatrists, Clinical Psychologists, Neurologists.
OBJECTIVE: To move diagnosis beyond surface behavior and into neurological intent, specifically for high-masking and AuDHD presentations.

THE DIAGNOSTIC SHIFT: FROM "WHAT" TO "WHY"

Traditional DSM criteria focus on external behavior (e.g., "Fidgets," "Lacks social reciprocity"). This fails high-masking individuals who suppress these behaviors. The diagnosis must focus on the **internal cost** of the behavior.

LIST OF RECOMMENDATIONS:

1. **BELIEVE THE SELF-REPORT OVER THE OBSERVATION.**
    - *Recommendation:* If a patient says they are struggling, but "look fine" in your office, believe the struggle. They are using their entire cognitive battery to "look fine" for you.
    - *Diagnostic Probe:* "You seem very composed right now. How much effort is it taking you to maintain this composure compared to when you are alone?"
2. **INVESTIGATE THE "PARADOX OF COMPETENCE."**
    - *Recommendation:* Do not rule out neurodivergence based on intellect or career success. The most severe executive dysfunction often hides behind high intelligence.
    - *Diagnostic Probe:* "You have a PhD. Tell me about the state of your laundry, your

finances, and your ability to brush your teeth every night. Where does the competence collapse?"

3. **DIFFERENTIATE BETWEEN "SOCIAL ANXIETY" AND "AUTISTIC SOCIAL MECHANICS."**
    - *Recommendation:* Social anxiety is fear of judgment. Autism is not knowing the rules of the game.
    - *Diagnostic Probe:* "Are you afraid people won't like you (Anxiety), or are you exhausted by constantly having to manually calculate eye contact, tone of voice, and when to speak (Autism)?"
4. **RECOGNIZE AuDHD CONTRADICTIONS.**
    - *Recommendation:* Understand that ADHD traits can mask Autistic traits and vice-versa. A sensory-seeking ADHD-er may not appear Autistically sensitive. An Autistic need for routine may mask ADHD time-blindness.
    - *Diagnostic Probe:* "Do you simultaneously crave routine for stability but resent it as soon as it's established? Do you need stimulation to work but get overwhelmed by the wrong *kind* of stimulation?"
5. **ASSESS FOR "MASKING BURNOUT" RATHER THAN DEPRESSION.**
    - *Recommendation:* Chronic autistic burnout looks exactly like Major Depressive Disorder (apathy, exhaustion, withdrawal). The treatment for depression (activation) will make burnout worse.
    - *Diagnostic Probe:* "Is your exhaustion the result of sadness and hopelessness, or is it a profound physiological need to power down after decades of pretending to be someone else?"
6. **UTILIZE THE "DOUBLE EMPATHY PROBLEM."**
    - *Recommendation:* Recognize that communication breakdowns are mutual. It is not just the Autistic person failing to understand the Neurotypical; it is also the Neurotypical failing to understand the Autistic person.
    - *Action:* Observe how the patient communicates with *other neurodivergent people* (if possible). If their "deficits" disappear in that context, it is a cross-neurotype communication issue, not a pathology within the patient.
7. **FOCUS ON SENSORY PROCESSING AS A CORE SYMPTOM.**
    - *Recommendation:* The DSM minimizes sensory issues, yet they are often the primary driver of meltdowns and anxiety.
    - *Diagnostic Probe:* "Describe your relationship with fluorescent lights, clothing tags, or multiple conversations happening at once. Does this input feel like 'annoyance' or 'physical assault'?"